

CANONICAL LESSON

V7_B_N14 - Evaluate What Changed in Health

Decision-centred foundations for accountable sector intelligence

Production Unit	PU-B07-C34
Notebook	V7_B_N14
Status	Controlled educational draft v0.1
Data boundary	Synthetic instructional data only

Central decision: Evaluate health change using explicit counterfactual logic, implementation evidence, equity, uncertainty, and decision relevance.

Learning outcomes

Why this matters

Before-and-after change alone cannot establish causation or guide scale-up safely.

The analytical workflow is useful only when evidence, uncertainty, authority, and action are connected explicitly. The model supports professional judgement; it does not replace accountable institutional authority.

Worked cross-sector example

A programme assessment separates observed change from plausible programme contribution.

Boundary: The example supports learning and review. It does not constitute an official statistic, eligibility determination, enforcement action, clinical judgement, or operational authorization.

Practice and reflection

- 1.
- State the identifying assumption.
- 2.
- Test differential attrition.
- 3.

- Separate impact and implementation failure.

Answer key

- 1.
- For difference-in-differences, untreated potential outcomes must follow comparable trends absent the programme.
- 2.
- Compare attrition by group and baseline risk and bound missing outcomes.
- 3.
- A null effect with low fidelity differs from a well-implemented ineffective intervention and calls for a different decision.

Operational readiness checklist

- Purpose and decision owner are named.
- Evidence definitions, provenance, and lawful access are documented.
- Uncertainty and abstention are visible.
- Human review, escalation, monitoring, incident response, and retirement are assigned.
- Equity, accessibility, security, and sector-specific approval are complete.